

## Supplementary Material

### The Neurotech EEG Dataset: A Large Clinical Scalp EEG Corpus Dominated by Multi-Day Ambulatory Recordings

Morgan et al.

#### Supplementary Tables

**Supplementary Table 1. Recording duration categories.** The majority of segments represent ambulatory or short-term monitoring.

| Category             | Duration     | N segments (%) | Likely clinical setting  |
|----------------------|--------------|----------------|--------------------------|
| Routine              | < 1 hour     | 8,545 (36%)    | Outpatient EEG           |
| Short monitoring     | 1 - 24 hours | 12,572 (53%)   | Ambulatory or short-term |
| Prolonged monitoring | > 24 hours   | 2,490 (11%)    | Multi-day ambulatory     |

*Percentages computed over 23,607 EDF segments containing recoverable signal data.*

**Supplementary Table 2. Annotation categories.** Technician clips and spike markers are the most frequent categories.

| Category                     | Events | Description                                  |
|------------------------------|--------|----------------------------------------------|
| Technician clips             | 53,469 | Segments selected for physician review       |
| Spike markers                | 50,482 | Interictal epileptiform discharge detections |
| Neurotech free-text comments | 24,267 | Free-text EEG finding descriptions           |
| Sharp waves                  | 21,330 | Sharp wave or sharp-slow-wave complexes      |
| Slowing                      | 19,401 | Focal or diffuse slowing                     |

|                           |        |                                            |
|---------------------------|--------|--------------------------------------------|
| Activation procedures     | 15,746 | Eyes open/closed, photic, hyperventilation |
| Generalized patterns      | 12,345 | Generalized discharges                     |
| Spike-wave                | 10,439 | Spike-wave complexes                       |
| Seizure markers           | 6,892  | Electrographic seizure events              |
| Posterior dominant rhythm | 5,535  | PDR frequency notations                    |
| Artifact                  | 4,881  | Technical or physiological artifacts       |
| Burst-suppression         | 3,350  | Burst-suppression pattern                  |
| Focal                     | 1,928  | Focal patterns                             |
| Normal                    | 505    | "Normal" notations                         |
| Epileptiform              | 164    | Other epileptiform                         |
| Periodic                  | 139    | Periodic discharges (LPDs/GPDs)            |

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13 *Categories are not mutually exclusive; a single annotation may contribute to multiple*  
14 *categories. Total annotation events: 226,486 across 14,517 segments with annotation*  
15 *files.*

16 **Supplementary Table 3. De-identification example.** Header field transformation and  
17 annotation scrubbing for a representative recording.

| Field                    | Original                                   | De-identified                    |
|--------------------------|--------------------------------------------|----------------------------------|
| Patient identification   | 12-34567 X 15-MAR-2023<br>Doe_Jane         | X X X X                          |
| Recording identification | Startdate 15-MAR-2023<br>Record_stopped... | Startdate 22-JUN-2023 X X X      |
| Start date               | 15.03.23                                   | 22.06.23 (shifted +99 days)      |
| Start time               | 14.32.08                                   | 14.32.08 (preserved)             |
| Annotation text          | Patient event Jane had a blank stare       | Patient event [NAME] had a blank |

|                      |                     |                               |
|----------------------|---------------------|-------------------------------|
|                      |                     | stare                         |
| Annotation timestamp | 2023-03-15T14:33:45 | 2023-06-22T14:33:45 (shifted) |

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19 *Names and dates shown are fictitious. Actual date shifts are random per patient (uniform*

20 *integer in [-365, +365] days) and applied consistently across all files for that patient.*

21 **Supplementary Table 4. Detailed EEG findings.** Breakdown of posterior dominant

22 rhythm (PDR), interictal epileptiform discharges, slowing, seizures, and patient-reported

23 events extracted from technologist scan reports (n=10,726 studies with extractable

24 reports).

| Finding                            | Value          | Notes                  |
|------------------------------------|----------------|------------------------|
| <b>Posterior dominant rhythm</b>   |                |                        |
| PDR extractable                    | 8057           | of 10726 studies       |
| PDR frequency, median (IQR)        | 9.0 (8.5-10.0) | Hz                     |
| Normal range (8-13 Hz)             | 6816           | (85%)                  |
| Slow (<8 Hz)                       | 1146           | (14%)                  |
| Interictal epileptiform discharges | 6345           | of 10726 studies (59%) |
| <b>Morphology</b>                  |                |                        |
| Spike                              | 3342           |                        |
| Sharp wave                         | 1755           |                        |
| Spike-and-wave                     | 1669           |                        |
| Polyspike                          | 502            |                        |
| <b>Distribution</b>                |                |                        |
| Generalized                        | 1691           |                        |
| Focal                              | 1364           |                        |

|                                                                    |                               |                  |
|--------------------------------------------------------------------|-------------------------------|------------------|
| Multifocal                                                         | 277                           |                  |
| Bilateral independent                                              | 97                            |                  |
| <b>Laterality</b>                                                  |                               |                  |
| Left / Right / Bilateral                                           | 1770 / 1611 / 709             |                  |
| <b>Region</b><br>(temporal/frontal/central/parietal/<br>occipital) | 1940 / 1261 / 878 / 329 / 404 |                  |
| Abnormal slowing                                                   | 4524                          | of 10726 studies |
| Electrographic seizures                                            | 2379                          | of 10726 studies |
| Patient-reported events                                            | 5119                          |                  |

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26 *Morphology, distribution, laterality, and region were extracted from free-text*

27 *descriptions using regex pattern matching. Categories are not mutually exclusive.*

28 **Supplementary Table 5. Hourly EEG monitoring data.** Summary of technician

29 monitoring activity extracted from monitoring logs in technologist scan reports.

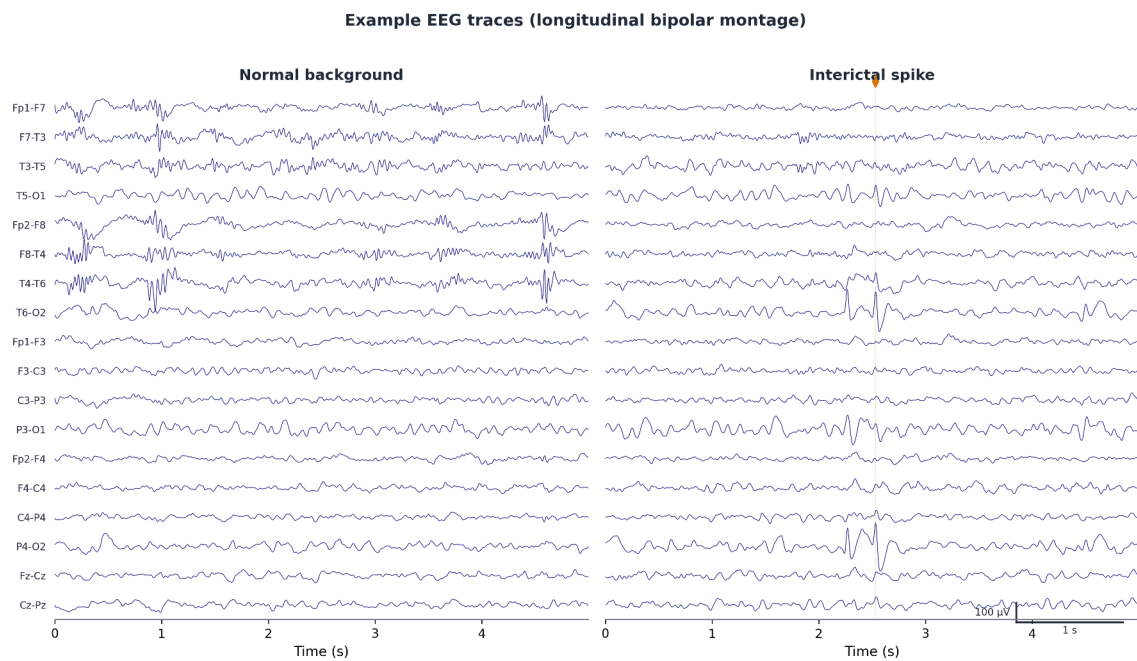
| <b>Metric</b>                                     | <b>Value</b>           |
|---------------------------------------------------|------------------------|
| Studies with monitoring data                      | 6413                   |
| Total logged hours                                | 296,948                |
| Hours recording active                            | 273,796 (92%)          |
| Distinct days/study, median (IQR)                 | 3.0 (2.0-4.0)          |
| Monitoring events                                 | 173,096                |
| EEG reviewed / General notes / Equipment failures | 134,875 / 37,704 / 517 |

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31 *Each monitoring hour was documented individually by the assigned technician, including*

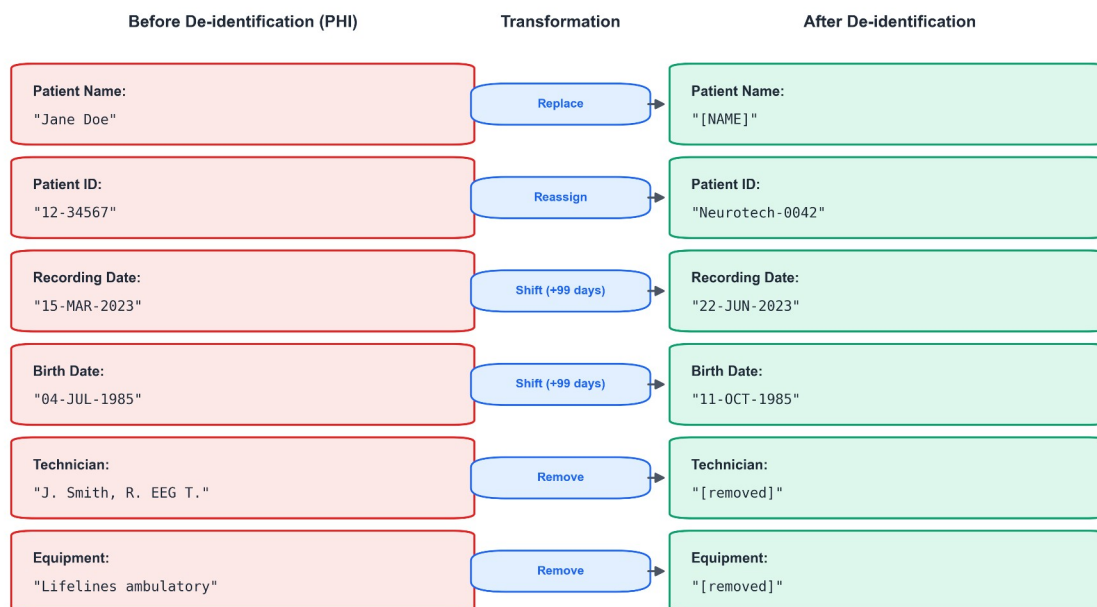
32 *impedance, recording status, battery level, and free-text observations.*

## 33    **Supplementary Figures**



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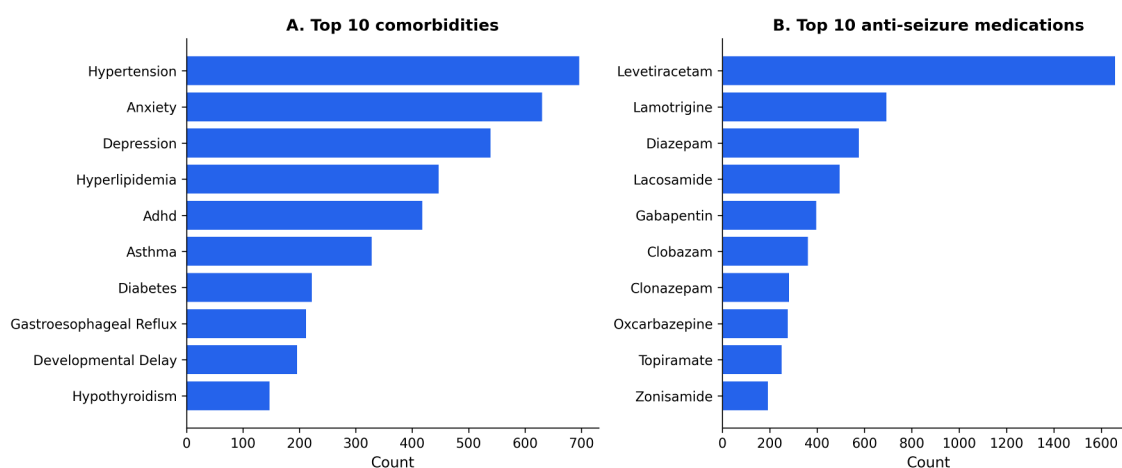
35    **Supplementary Figure 1.** Example EEG traces from a representative recording in  
36    standard bipolar montage showing normal background activity (left) and an interictal  
37    spike (right). Channels from the standard 10-20 montage are displayed; scale bars  
38    indicate 100 microvolts and 1 second.



Recording and birth dates are shifted by the same random per-patient offset; times of day are preserved. All examples are fictitious.

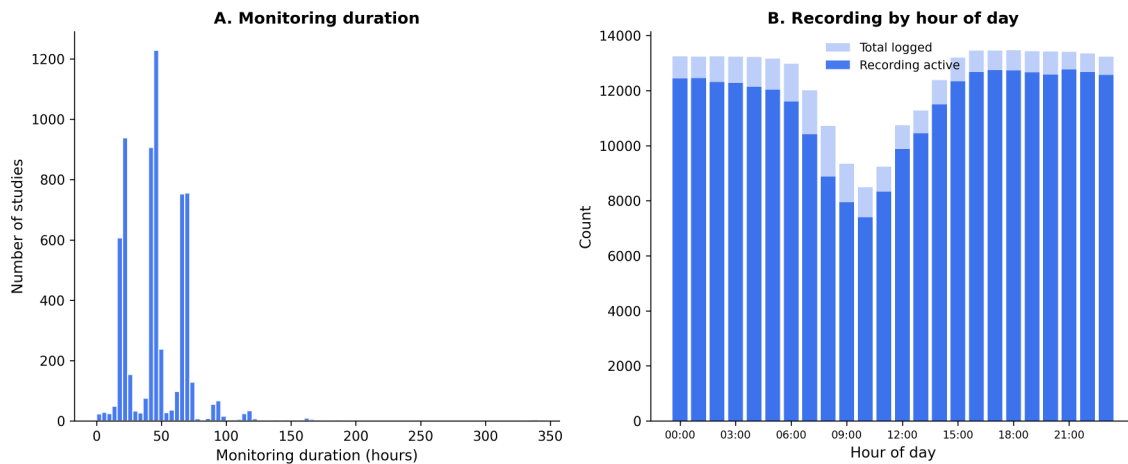
39

40 **Supplementary Figure 2.** De-identification of EDF header fields. Each row shows a  
 41 header field before (containing protected health information) and after de-identification.  
 42 Patient names are replaced with placeholders, identifiers are reassigned, dates are shifted  
 43 by a consistent per-patient random offset, and technician and equipment identifiers are  
 44 removed. All examples shown are fictitious.



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46 **Supplementary Figure 3.** Comorbidities and anti-seizure medications. (A) Ten most  
 47 frequent comorbidities extracted from clinical progress notes, excluding seizure-related  
 48 diagnoses. (B) Ten most frequently prescribed anti-seizure medications. Levetiracetam is  
 49 the most common, consistent with its status as a first-line agent.



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51 **Supplementary Figure 4.** Monitoring characteristics. (A) Distribution of total  
 52 monitoring duration across 6413 studies with hour-by-hour monitoring data. Peaks at 24  
 53 and 48 hours reflect standard ordered monitoring durations. (B) Recording activity by  
 54 hour of day across all 296,948 documented monitoring hours. Light shading shows total  
 55 hours logged; dark shading shows hours with active recording.